AR 2EB

ALAMEDA RESEARCH
2: ELECTRIC BOOGALOO
LONG HORIZONS · STRUCTURAL SHIFTS · IMAGINATION

INTERNAL RESEARCH · MEMO · NOT INVESTMENT ADVICE · AI-ASSISTED

GRAIL

NASDAQ : GRAL · Young-Company DCF (Damodaran)

$5.83B mkt cap · 44M sh · $823M cash, zero debt · $0.823B cash (Q1'26), no debt; ~44M shares (low float); Illumina ~14.5% at the 2024 spin; Revenue-generating: $147M FY25 (+17%), Galleri self-pay ~$949; burn ~$300M/yr (~2.5yr runway); NHS-Galleri missed primary endpoint (Feb'26); FDA PMA accepted; Medicare law (≥2028, ~$509 parity); Abbott bought Exact

22 October 2026 · the qualitative

$132.60

THE CENTRAL QUESTION

Does ~$5.8B / ~39x sales price GRAIL's multi-gate regulatory option fairly — after the NHS-Galleri miss, but with the FDA filing accepted and the Medicare law in hand?

GRAIL trades at ~$5.8B (≈$133/sh on just ~44M shares) on ~$147M FY25 revenue and ~$823M cash. Galleri — a blood test for 50+ cancers, self-pay today — is a multi-gate option: FDA PMA (filed Jan 2026, now accepted), then Medicare coverage (signed Feb 2026; ≥2028 at ~$509 parity), then broad adoption. The pivotal NHS-Galleri trial MISSED its primary endpoint (-50% stock), but the PMA rests on positive PATHFINDER 2 and the law passed — broken yet de-risked. Re-modeled on ~$509 parity (Abbott now backs Cancerguard), at ~$133 the market prices well above the modal regulatory-success case — it needs the broad-adoption upside.

PROBABILITY-WEIGHTED EXPECTED VALUE

most likely case (base) -56.0% · 82% at or below spot

$65.99

expected fair value today

-50.2% vs spot $132.60

FORWARD COMPOUNDED VALUE

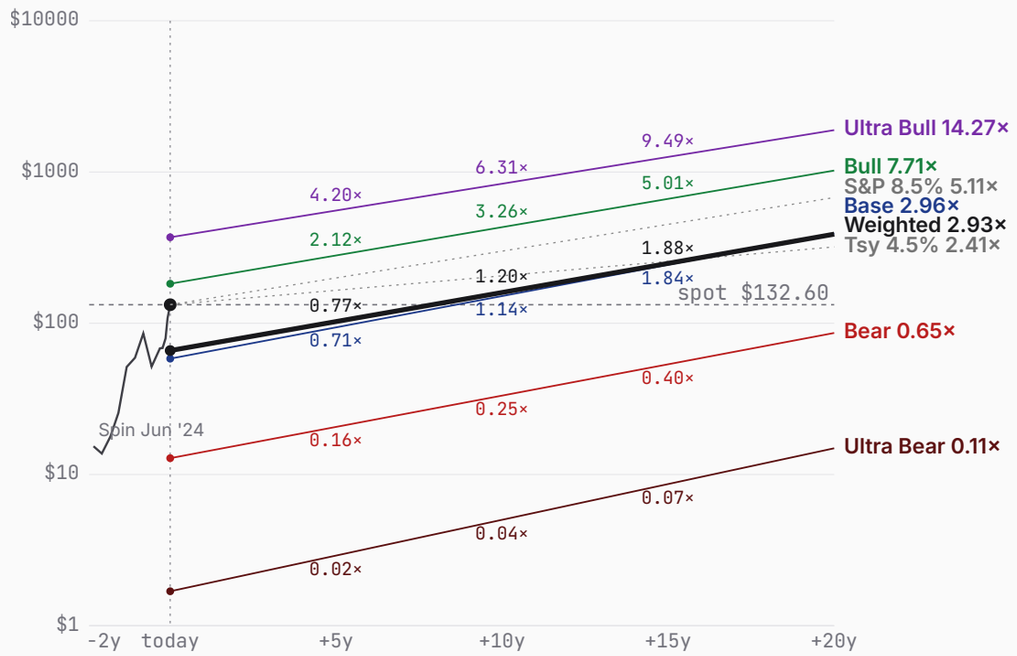
+5y	+10y	+15y	+20y
$102.64	$159.77	$248.91	$388.13
0.77× spot	1.20× spot	1.88× spot	2.93× spot

WEIGHTING RATIONALE

Ultra Bear 20% FDA rejects; no coverage; near-total loss.
Bear 28% FDA late; Medicare narrow; sub-scale; low teens.
Base 34% FDA 2027 + Medicare 2028; leader; -56%.
Bull 13% Broad adoption; category lead; ~1.4×.
Ultra Bull 5% MCED standard of care; dominant; <3×.

Bull (13%) + Ultra Bull (5%) together contribute $42.23 — 64% of the $65.99 expected value despite 18% combined probability. Today's spot ($132.60) sits 101% above the weighted expected; the forward-weighted value reaches spot in about 8 years.

PRICE + PROJECTIONS · HISTORICAL THROUGH PROBABILITY-WEIGHTED FORWARD



ULTRA BEAR	BEAR	BASE	BULL	ULTRA BULL
$1.69	$12.82	$58.34	$182.48	$370.13
-98.7% vs spot	-90.3% vs spot	-56.0% vs spot	+37.6% vs spot	+179.1% vs spot
TAM 1.3% P(fail) 40% S2C 1.0 Prob 20%	TAM 4.1% P(fail) 22% S2C 1.1 Prob 28%	TAM 8.2% P(fail) 12% S2C 1.3 Prob 34%	TAM 14.4% P(fail) 6% S2C 1.5 Prob 13%	TAM 22.4% P(fail) 3% S2C 1.7 Prob 5%
FDA rejects; no coverage; self-pay niche.	FDA late; Medicare narrow; modest volume.	FDA 2027 + Medicare 2028; category leader.	Broad adoption; annual-repeat screening.	MCED standard of care; GRAIL dominant.

NOT INVESTMENT ADVICE · Not from a registered investment advisor · AI-assisted analysis · Author may hold positions · See full disclaimers, page 7

AR 2EB | ALAMEDA RESEARCH 2: ELECTRIC BOOGALOO — LONG HORIZONS · STRUCTURAL SHIFTS · IMAGINATION

GRAIL scenario narratives

PROBABILITY WEIGHTS

Ultra Bear 20% Bear 28% Base 34% Bull 13% Ultra Bull 5% · Weighted expected $65.99 (-50.2% vs spot)

ULTRA BEAR $1.69

-98.7% vs spot

Probability 20%

FDA rejects; no coverage; self-pay niche.

WHY 20%

The NHS-Galleri miss is a real, fired datapoint — the only RCT of any MCED failed its stage-shift primary endpoint, hardening FDA and payer skepticism. 40% within-scenario failure reflects genuine PMA-rejection risk. 20% scenario weight.

WHAT HAPPENS

The FDA PMA is rejected or stalled — the NHS-Galleri primary-endpoint miss (no significant stage III+IV reduction) hardens FDA/USPSTF skepticism about clinical utility, and Galleri stays a self-pay LDT niche. Test volume plateaus, the Medicare benefit never activates (it is gated on FDA approval), and GRAIL burns its ~$823M cash defending the franchise.

With a 40% within-scenario failure probability and revenue stalled near $0.3B, the equity collapses toward the cash-and-distress floor — a near-total loss vs today's $133.

BEAR $12.82

-90.3% vs spot

Probability 28%

FDA late; Medicare narrow; modest volume.

WHY 28%

FDA approves but adoption disappoints — Medicare narrow, private payers await mortality proof, ASP compresses. 22% failure probability. 28% weight as a very plausible 'approved but sub-scale' outcome.

WHAT HAPPENS

The FDA approves but late, and Medicare coverage arrives narrow and slow; private payers wait for the mortality data the NHS trial did not deliver. Galleri reaches modest reimbursed volume (~$1B revenue by 2035) at thin-but-improving margins — a real test, sub-scale economics.

The DCF lands in the low teens — well below spot. This is the 'approved, but adoption disappoints' world the NHS miss makes plausible.

BASE $58.34

-56.0% vs spot

Probability 34%

FDA 2027 + Medicare 2028; category leader.

WHY 34%

Requires FDA approval (on PATHFINDER 2, which was positive) + Medicare activation (the law is already signed) + broad coverage — the machinery is in motion. 12% failure probability. 34% weight as the central, modal outcome.

WHAT HAPPENS

The modal path: the FDA approves Galleri (~2027) on the PATHFINDER 2 evidence, Medicare coverage activates (~2028), and broad private coverage follows — Galleri becomes the leading reimbursed MCED test at ~$2.05B revenue by 2035 (~8% of a ~$25B screening market) at ~30% operating margins, blended ASP settling toward the ~$509 Medicare parity rate.

DCF ~$65/share — well below today's ~$133. The market prices above this modal regulatory-success case; the small ~50M share count makes the per-share value swing hard on each gate.

BULL $182.48

+37.6% vs spot

Probability 13%

Broad adoption; annual-repeat screening.

WHY 13%

A chain: clean FDA + Medicare + broad private coverage + annual-repeat adoption + category leadership. Each plausible at 50-70%; jointly ~13%. GRAIL's breadth and only-RCT dataset are the edge.

WHAT HAPPENS

The bull: FDA + Medicare clear cleanly, broad private coverage and annual-repeat screening take hold, and GRAIL extends its first-mover, broadest-test, only-RCT-dataset lead — ~$3.6B revenue by 2035 at ~36% operating margins. The screening category compounds and GRAIL leads it even as ASP holds near parity.

DCF ~$193/share — ~1.5× spot. This is the scenario the regulatory machinery (PMA accepted + Medicare law) keeps alive despite the NHS miss.

ULTRA BULL $370.13

+179.1% vs spot

Probability 5%

MCED standard of care; GRAIL dominant.

WHY 5%

Tail of tails: MCED standard of care + eventual mortality proof + GRAIL dominance + premium terminal. Individually 40-60%; jointly ~5%. The asymmetric upside the ~43M share count amplifies.

WHAT HAPPENS

The tail: MCED becomes standard of care, the extended NHS follow-up eventually shows a mortality benefit, and GRAIL is the dominant platform — ~$5.6B revenue by 2035 at ~40% margins with a premium terminal.

DCF $381/share — 2.9× spot. The asymmetric upside the low share count amplifies; individually each gate is plausible, jointly ~5%.

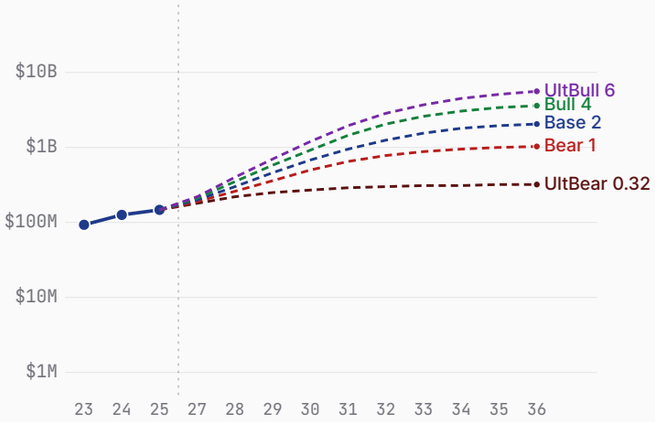
NOT INVESTMENT ADVICE · Not from a registered investment advisor · AI-assisted analysis · Author may hold positions · See full disclaimers, page 7

GRAIL business snapshot

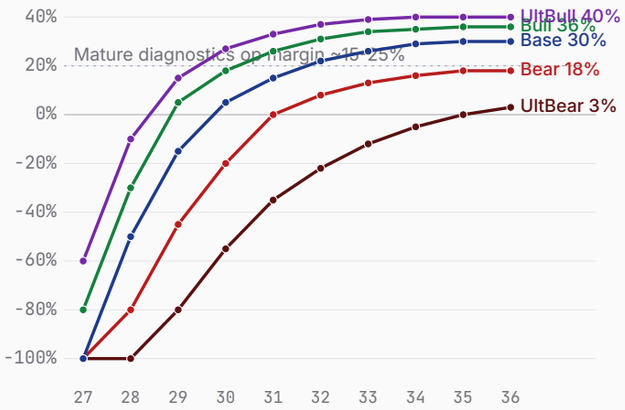
Bear $12.82 Base $58.34 Bull $182.48

FY26-FY35 scenario projections (revenue-generating; pre-profit) · fiscal years end Dec 31 · Q1 2026 10-Q, FY25 results

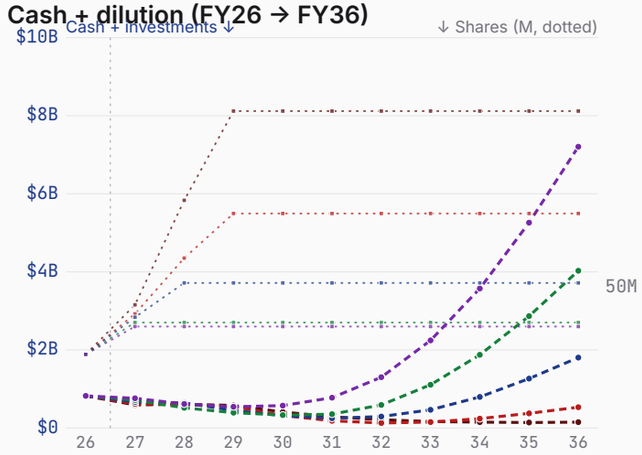
Revenue — history + scenarios to FY36 (log)



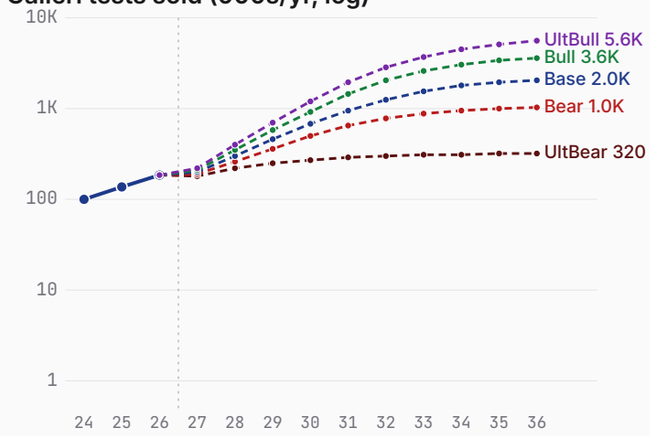
Path to profitability — op margin (FY27→FY36)



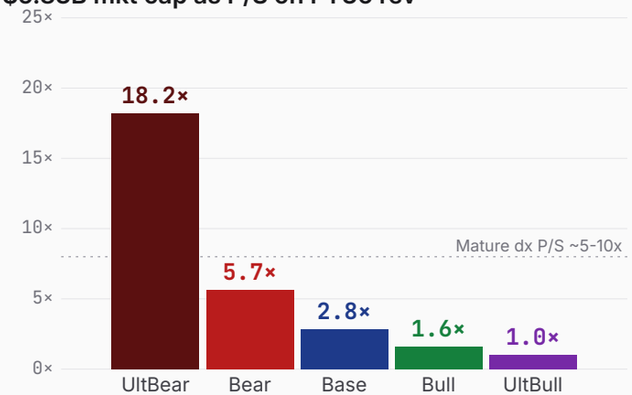
Cash + dilution (FY26 → FY36)



Galleri tests sold (000s/yr, log)

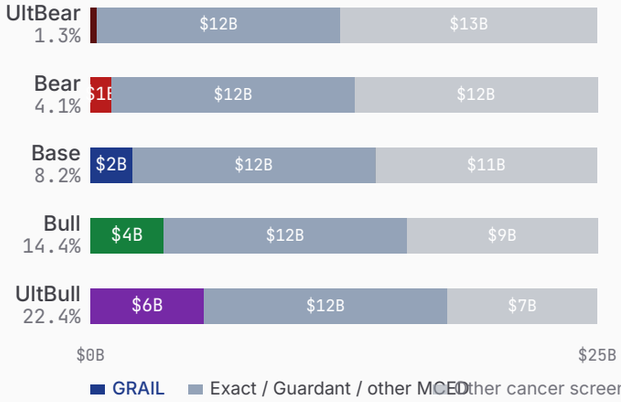


$5.83B mkt cap as P/S on FY36 rev



At ~39x sales today, every scenario implies the multiple resolves only as reimbursed test volume arrives.

$25B FY35 MCED screening TAM — GRAIL share



Sources: GRAIL Q1 2026 10-Q (cash $0.823B), FY2025 results ($147M revenue, net loss), NHS-Galleri (primary endpoint missed Feb'26) + PATHFINDER 2 (positive) readouts, FDA PMA (accepted), Medicare MCED Coverage Act (≥2028, ~$509 parity). Revenue modeled as reimbursed test volume x ASP compressing toward ~$509; FCF NOL-shielded pre-tax (documented simplification). TAM: MCED screening slice ~$25B by FY35 (GRAIL low-teens share).

NOT INVESTMENT ADVICE · Not from a registered investment advisor · AI-assisted analysis · Author may hold positions · See full disclaimers, page 7

GRAIL 7 Powers · the moat, scored

Bear $12.82 Base $58.34 Bull $182.48

Arena. Multi-cancer early detection (MCED) blood screening — GRAIL (Galleri, 50+ cancers, only RCT) vs Abbott/Exact (Cancerguard + Cologuard cash engine), Guardant (Shield, FDA-approved CRC → MCED), Freenome, Delfi; the race to originate the first FDA-approved, reimbursed MCED franchise.

POWER ORIGINATION
origination window: open

POWER ORIGINATION · 7 POWERS

Scale Economies	A reimbursed-volume cost curve is future; today's self-pay volume is sub-scale.
Network Economies	No network flywheel yet; data scale is the closest analog.
Counter-Positioning	Galleri's 50-cancer breadth + build-own dataset is hard for single-cancer (Shield CRC) entrants to mirror without their own large trials.
Switching Costs	Provider-ordering and EHR integration build mild lock-in; early.
Branding	The MCED category-definer brand; strong but dented by the NHS miss.
Cornered Resource · thesis leans here	The only randomized MCED dataset (NHS ~140k) + first PMA + largest commercial test base — the strongest, if contested, origination asset.
Process Power	Assay + bioinformatics pipeline is real but not a demonstrated durable process edge.

THE RACE · NAMED RIVALS

Abbott / Exact Sciences — Abbott-backed; global scale

~27% terminal share · Cancerguard MCED (LDT, launched Sep'25, $689) now owned by Abbott (deal closed Mar'26) — a 160-country distributor behind the Cologuard franchise.

Guardant Health — ~$700M-rev co.

~22% terminal share · Shield FDA-approved for CRC (Medicare $1,495); extending to MCED (Breakthrough Device).

Freenome — $1.35B private

~10% terminal share · CRC-first → multi-cancer; ~$1.35B raised (Roche, T. Rowe).

Delfi Diagnostics — Merck GHI + rounds

~5% terminal share · FirstLook Lung (fragmentomics); niche, lung-first.

LEAD / LAG · FALSIFIERS

MCED clinical evidence (RCT)	NHS ~140k (only RCT; primary missed) vs none randomized	LEADING
FDA approval	PMA accepted (decision 2026-27) vs Guardant Shield approved (CRC)	ROUGHLY LEVEL
Reimbursement / commercial scale	self-pay; Medicare gated to 2028 vs Exact Cologuard reimbursed engine	LAGGING

COMPETITIVE READ

GRAIL is originating the MCED category — the broadest test (50+ cancers), the only randomized dataset, and the first PMA — but the NHS-Galleri primary-endpoint miss dented the 'proven population benefit' narrative, and Abbott/Exact (Cancerguard on a 160-country distributor) and Guardant (FDA + Medicare in CRC) are scaling MCED behind it. The origination window is open but closing: GRAIL must convert the PMA into approval + Medicare coverage before better-capitalized rivals scale, and the extended NHS mortality follow-up is the falsifier that could reopen — or close — the whole thesis.

NOT INVESTMENT ADVICE · Not from a registered investment advisor · AI-assisted analysis · Author may hold positions · See full disclaimers, page 7

GRAIL show your work

Bear $12.82 Base $58.34 Bull $182.48 · Weighted $65.99 (-50.2%)

ULTRA BEAR $1.69

ASSUMPTIONS

TAM Year-10 ($B)	25
Mature share (%)	1.3
Mature op margin (%)	3
Sales-to-capital	1.0
Terminal WACC (%)	11.5
Terminal growth (%)	2.0
P(failure) (%)	40
Scenario probability (%)	20

10-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY27	0.18	-140%	-0.28	-0.25
FY28	0.22	-110%	-0.28	-0.21
FY29	0.25	-80%	-0.23	-0.15
FY30	0.27	-55%	-0.17	-0.10
FY31	0.29	-35%	-0.12	-0.06
FY32	0.30	-22%	-0.08	-0.04
FY33	0.31	-12%	-0.05	-0.02
FY34	0.31	-5%	-0.01	-0.01
FY35	0.32	+0%	-0.01	-0.00
FY36	0.32	+3%	+0.01	+0.00

Σ PV explicit FCF	**-0.84**
+ PV terminal (g 2.0%)	0.03

EQUITY BUILD · $B & PER SHARE

Operating EV	$-0.81B
+ Cash & investments	$0.82B
= Equity value	**$0.01B**
Raised over period	$0.70B
Dilution by FY36	+50%
FY36 shares (M)	66
DCF per share	**$0.15**
× (1−P_fail) [60%]	$0.09
+ P_fail × distress [40% × $4.00]	$1.60

= Expected per share $1.69

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$2.91	$5.02	$8.65	$14.91
0.02×	0.04×	0.07×	0.11×

BEAR $12.82

ASSUMPTIONS

TAM Year-10 ($B)	25
Mature share (%)	4.1
Mature op margin (%)	18
Sales-to-capital	1.1
Terminal WACC (%)	10.0
Terminal growth (%)	2.5
P(failure) (%)	22
Scenario probability (%)	28

10-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY27	0.19	-120%	-0.27	-0.23
FY28	0.26	-80%	-0.27	-0.21
FY29	0.36	-45%	-0.25	-0.17
FY30	0.50	-20%	-0.23	-0.14
FY31	0.65	+0%	-0.14	-0.07
FY32	0.78	+8%	-0.06	-0.03
FY33	0.88	+13%	+0.02	+0.01
FY34	0.95	+16%	+0.09	+0.03
FY35	1.00	+18%	+0.14	+0.05
FY36	1.03	+18%	+0.16	+0.05

Σ PV explicit FCF	**-0.71**
+ PV terminal (g 2.5%)	0.70

EQUITY BUILD · $B & PER SHARE

Operating EV	$-0.01B
+ Cash & investments	$0.82B
= Equity value	**$0.81B**
Raised over period	$0.65B
Dilution by FY36	+27%
FY36 shares (M)	56
DCF per share	**$14.46**
× (1−P_fail) [78%]	$11.28
+ P_fail × distress [22% × $7.00]	$1.54

= Expected per share $12.82

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$20.65	$33.25	$53.55	$86.25
0.16×	0.25×	0.40×	0.65×

BASE $58.34

ASSUMPTIONS

TAM Year-10 ($B)	25
Mature share (%)	8.2
Mature op margin (%)	30
Sales-to-capital	1.3
Terminal WACC (%)	10.0
Terminal growth (%)	2.5
P(failure) (%)	12
Scenario probability (%)	34

10-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY27	0.20	-100%	-0.24	-0.21
FY28	0.30	-50%	-0.23	-0.18
FY29	0.46	-15%	-0.19	-0.13
FY30	0.68	+5%	-0.14	-0.08
FY31	0.95	+15%	-0.07	-0.04
FY32	1.25	+22%	+0.04	+0.02
FY33	1.55	+26%	+0.17	+0.08
FY34	1.80	+29%	+0.33	+0.14
FY35	1.95	+30%	+0.47	+0.17
FY36	2.05	+30%	+0.54	+0.18

Σ PV explicit FCF	**-0.06**
+ PV terminal (g 2.5%)	2.48

EQUITY BUILD · $B & PER SHARE

Operating EV	$2.42B
+ Cash & investments	$0.82B
= Equity value	**$3.24B**
Raised over period	$0.40B
Dilution by FY36	+14%
FY36 shares (M)	50
DCF per share	**$64.80**
× (1−P_fail) [88%]	$57.02
+ P_fail × distress [12% × $11.00]	$1.32

= Expected per share $58.34

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$93.96	$151.32	$243.70	$392.48
0.71×	1.14×	1.84×	2.96×

BULL $182.48

ASSUMPTIONS

TAM Year-10 ($B)	25
Mature share (%)	14.4
Mature op margin (%)	36
Sales-to-capital	1.5
Terminal WACC (%)	9.0
Terminal growth (%)	3.0
P(failure) (%)	6
Scenario probability (%)	13

10-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY27	0.21	-80%	-0.21	-0.19
FY28	0.35	-30%	-0.20	-0.16
FY29	0.58	+5%	-0.12	-0.09
FY30	0.92	+18%	-0.06	-0.04
FY31	1.45	+26%	+0.02	+0.01
FY32	2.05	+31%	+0.24	+0.12
FY33	2.60	+34%	+0.52	+0.24
FY34	3.05	+35%	+0.77	+0.33
FY35	3.40	+36%	+0.99	+0.39
FY36	3.60	+36%	+1.16	+0.42

Σ PV explicit FCF	**+1.05**
+ PV terminal (g 3.0%)	7.20

EQUITY BUILD · $B & PER SHARE

Operating EV	$8.25B
+ Cash & investments	$0.82B
= Equity value	**$9.07B**
Raised over period	$0.20B
Dilution by FY36	+7%
FY36 shares (M)	47
DCF per share	**$192.98**
× (1−P_fail) [94%]	$181.40
+ P_fail × distress [6% × $18.00]	$1.08

= Expected per share $182.48

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$280.77	$432.00	$664.68	$1,022.69
2.12×	3.26×	5.01×	7.71×

ULTRA BULL $370.13

ASSUMPTIONS

TAM Year-10 ($B)	25
Mature share (%)	22.4
Mature op margin (%)	40
Sales-to-capital	1.7
Terminal WACC (%)	8.5
Terminal growth (%)	3.0
P(failure) (%)	3
Scenario probability (%)	5

10-YEAR DCF · $B

FY	Rev	Margin	FCF	PV FCF
FY27	0.22	-60%	-0.17	-0.16
FY28	0.40	-10%	-0.15	-0.12
FY29	0.70	+15%	-0.07	-0.05
FY30	1.20	+27%	+0.03	+0.02
FY31	1.95	+33%	+0.20	+0.12
FY32	2.85	+37%	+0.53	+0.29
FY33	3.70	+39%	+0.94	+0.47
FY34	4.50	+40%	+1.33	+0.60
FY35	5.10	+40%	+1.69	+0.71
FY36	5.60	+40%	+1.95	+0.75

Σ PV explicit FCF	**+2.63**
+ PV terminal (g 3.0%)	14.06

EQUITY BUILD · $B & PER SHARE

Operating EV	$16.69B
+ Cash & investments	$0.82B
= Equity value	**$17.51B**
Raised over period	$0.20B
Dilution by FY36	+5%
FY36 shares (M)	46
DCF per share	**$380.65**
× (1−P_fail) [97%]	$369.23
+ P_fail × distress [3% × $30.00]	$0.90

= Expected per share $370.13

FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT

+5y	+10y	+15y	+20y
$556.55	$836.86	$1,258.35	$1,892.12
4.20×	6.31×	9.49×	14.27×

NOT INVESTMENT ADVICE · Not from a registered investment advisor · AI-assisted analysis · Author may hold positions · See full disclaimers, page 7

GRAIL People · Opportunity · Context · Deal

Bear $12.82 Base $58.34 Bull $182.48

Underwrite the position the way a venture investor underwrites a round — across all four legs, public or private. **People** is the leg scored here (observable only); Opportunity, Context and Deal cross-reference the rest of the memo.

PEOPLE · WHO RUNS / OWNS / FUNDS IT

Josh Ofman — 0 yrs in seat

1.8%
insider ownership

MEDIUM
key-person risk

Capital allocation (realized)
GRAIL is pre-profit and cash-burning. It was spun out of Illumina in June 2024 (Illumina had paid ~$8B in 2021; the public market now values GRAIL near ~$5.8B — heavy value destruction under prior ownership, pre-spin). Current management has roughly halved the cash burn to ~$300M/yr and holds ~$823M cash with no debt, funding the company into the ~2028 Medicare window without an imminent raise. FY25 revenue $147M (+17%). No dividend or buyback — appropriate for a pre-scale development-stage screener.

Incentive alignment
CEO pay is predominantly equity (RSU grants per Form 4 filings); Ofman directly owns ~0.4M shares (~1%). Total insider ownership is low (~1.8%) — typical of a corporate spinco and weaker skin-in-the-game than a founder-led name, with no founder block on the register.

GOVERNANCE FLAGS

- single class of common stock, one vote per share — no dual-class or super-voting
- independent Chair (Greg Summe) is separate from the CEO — the Chair and CEO roles are split
- brand-new CEO: Josh Ofman became CEO 1 June 2026 (Bob Ragusa retired); ~7 years at GRAIL prior (CMO/President since 2019), but under a month in the top seat during the post-NHS-miss period
- pending securities class-action litigation following the NHS-Galleri primary-endpoint miss (lead-plaintiff deadline Aug 2026)
- Illumina retained ~14.5% at the June-2024 spin (a diminishing overhang as it divests); the franchise carries the history of ~$8B destroyed under Illumina ownership in 2021-2024 (pre-spin)

THE FOUR LEGS

People — observable composite · 1–5



Opportunity
The scenario distribution · the business snapshot

Context
The 7 Powers analysis (Power Origination)

Deal
Open-market common — entry price weighed against the probability-weighted fair value, then position-sized.
-50.2% vs spot $132.60

PEOPLE READ

Professional management on a clean single-class register with an independent Chair (Summe) split from the CEO — but a brand-new CEO (Ofman, June 2026), thin insider ownership (~1.8%, no founder block), a fresh post-NHS securities class action, and the spinco history of ~$8B destroyed under Illumina hold it at a middling 3; the offsets are the split Chair/CEO, the disciplined ~$823M cash runway, and the absence of dual-class.

NOT INVESTMENT ADVICE · Not from a registered investment advisor · AI-assisted analysis · Author may hold positions · See full disclaimers, page 7

GRAIL supporting analysis · disclaimers · glossary

Bear $12.82 Base $58.34 Bull $182.48

PUSHBACK · WHY THE BASE CASE IS TOO HARSH

1 **Only randomized MCED dataset.**
The NHS-Galleri RCT (~140k) is the largest, only randomized MCED dataset — a cornered resource even after the primary-endpoint miss (stage IV fell -14%; mortality follow-up extended).

2 **FDA filing rests on PATHFINDER 2, not NHS.**
The PMA is built on the positive PATHFINDER 2 (69.8% sensitivity for the 12 deadliest, 99.6% specificity) plus the NHS prevalent round — not the failed late-stage primary.

3 **Medicare law already signed.**
The MCED coverage benefit is law (Feb 2026); coverage is gated on FDA approval and earliest 2028 at ~$509 mt-stool-DNA parity, but the legislative risk is retired.

4 **Cash-funded into the coverage window.**
~$823M and burn halved to ~$300M/yr funds GRAIL into the ~2028 Medicare window without an imminent raise.

5 **Tiny share count amplifies the unlock.**
~43M shares means a reimbursed-volume inflection moves per-share value hard — the asymmetry cuts both ways.

FALSIFICATION TRIGGERS

Bull validation
FDA PMA approval (2026-27) · Medicare NCD activates (~2028) · broad private coverage · test volume sustains >40% growth

Bear validation
PMA rejected/delayed · Medicare narrow/post-2028 or stuck at ~$509 parity · Abbott-backed Cancerguard scales · private payers demand mortality data

Reframe needed
if the extended NHS follow-up shows a mortality benefit, the clinical-utility debate resolves and the bull/ultra weights rise materially

DISCLAIMERS · PLEASE READ BEFORE USING THIS DOCUMENT

Not investment advice.
This document is produced for entertainment and educational purposes only by an individual amateur investor. "Alameda Research 2: Electric Boogaloo" is the unincorporated personal concept of one person and is not a registered investment management entity. Nothing herein constitutes investment advice, a recommendation, or a solicitation to buy or sell any security.

Not a professional.
The author is not a registered investment advisor, broker-dealer, CFA charterholder, certified financial planner, or licensed financial professional of any kind. The author has no formal training in equity research or capital markets. No fiduciary relationship is created by reading this document.

AI-assisted analysis.
Substantial portions of this analysis were produced with the assistance of large language models. LLMs are known to fabricate facts, miscalculate numbers, hallucinate sources, and present incorrect information with high apparent confidence. Every figure, claim, and projection in this document should be independently verified before acting on it.

Forward-looking statements.
Scenarios, valuations, projections, and any forward-looking statements involve substantial assumptions and uncertainty. Actual results may differ materially from those projected. Past performance is not indicative of future results. The model is a model; reality is not.

Author may hold positions.
The author may own, intend to acquire, or hold short exposure to $GRAL or related securities at any time. Positions may change without notice and without an update to this document. Do not assume the author's positions match the tone of the analysis.

Do your own research.
Do not make investment decisions on the basis of this document. Consult a licensed financial professional and read the company's official SEC filings (10-K, 10-Q, 8-K, proxy) before forming any investment view. If you wouldn't trust your retirement to a chatbot, don't trust this either.

GLOSSARY · CONCEPTS REFERENCED IN THE NARRATIVE

Galleri — GRAIL's blood test screening for 50+ cancers from one draw; sold self-pay (~$949) today as an LDT, FDA PMA filed Jan 2026 (accepted).

MCED — Multi-cancer early detection — blood-based screening across many cancer types; a new category GRAIL pioneered.

NHS-Galleri — The ~140k-participant UK randomized trial; missed its stage-shift primary endpoint (Feb 2026) but cut stage IV; mortality follow-up extended.

PMA / NCD — FDA Premarket Approval (filed Jan 2026, now accepted) and Medicare National Coverage Determination (earliest 2028, ~$509 mt-stool-DNA parity through 2030) — the two gates to reimbursed scale.

Sales-to-capital (s2c) — Damodaran reinvestment efficiency — incremental revenue per dollar of reinvested capital.